

23SMCV03434 23SMCV03434

County: los-angeles

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TENTATIVE

RULING

DEPARTMENT

207

HEARING DATE

July

29, 2026

CASE NUMBER

25SMCV03434

MOTION

Demurrer

MOVING PARTIES

Defendants

Dr. Rodney R. Raanan, DDS, MMSC; Rodney Raanan DDS Inc.; Raanan Dental Group
Beverly Hills Inc.; Raanan Dental Group Brentwood Inc.; and Little Smiles of
Beverly Hills Dental Office of Nassiripour DDS Inc.

OPPOSING PARTY

Plaintiff

Nefertiti Abdou

MOTION

This case arises from allegations of dental malpractice on reconstruction work done to correct injuries resulting from another dentist's malpractice.

On July 8, 2025, Plaintiff Nefertiti Abdou ("Plaintiff") filed suit against Defendants Dr. Rodney R. Raanan, DDS, MMSC; Rodney Raanan DDS Inc.; Raanan Dental Group Beverly Hills Inc.; Raanan Dental Group Brentwood Inc.; and Little Smiles of Beverly Hills Dental Office of Nassiripour DDS Inc. ("Defendants") alleging three causes of action for (1) medical malpractice; (2) negligent infliction of emotional distress; and (3) breach of fiduciary duty.

Defendants now demur to all three causes of action on the grounds that they fail to state facts sufficient to constitute a cause of action and uncertainty, pursuant to Code of Civil Procedure section 430.10, subdivisions (e) and (f), respectively.

Plaintiff opposes the demurrer and Defendants reply.

REQUEST

FOR JUDICIAL NOTICE

Plaintiff requests the Court to take judicial notice of the following:

1. Plaintiff's Verified Complaint filed July 8, 2025, in this action (Case No. 25SMCV03434).
2. Declaration of Rodney R. Raanan, DDS, in Support of Defendants' Motion to Set Aside/Vacate Entry of Default, filed December 4, 2025, in this action.
3. Declaration of Robert W. Harrison in Support of Defendants' Motion to Set Aside/Vacate Entry of Default, filed December 4, 2025, in this action.

4. Plaintiff's Declaration in Support of
Opposition to Defendants' Motion to Set Aside/Vacate Entry of Default, filed
April 15, 2026, in this action, including all exhibits attached thereto.

5. The Court's April 28, 2026 Minute Order on
Defendants' Motion to Set Aside/Vacate Entry of Default in this action.

Judicial notice may be taken
of records of any court in this state.
(Evid. Code, § 452, subd. (d)(1).)

Because the above documents are part of the Court's record for this
case, the Court may take judicial notice of them. (Ibid.) However, "while courts are free to take
judicial notice of the existence of each document in a court file, including
the truth of results reached, they may not take judicial notice of the truth of
hearsay statements in decisions and court files. Courts may not take judicial notice of
allegations in affidavits, declarations and probation reports in court records
because such matters are reasonably subject to dispute and therefore require
formal proof." (Lockley v. Law Office
of Cantrell, Green, Pekich, Cruz & McCort (2001) 91 Cal.App.4th 875,
882 [cleaned up].)

Accordingly, the Court takes
judicial notice of the existence, filing, and legal consequences of the above
documents filed in this matter as court records, but not the truth of the
allegations or other hearsay statements contained therein.

ANALYSIS

1.
DEMURRER

"It is black letter law that
a demurrer tests the legal sufficiency of the allegations in a complaint."
(Lewis v. Safeway, Inc. (2015) 235 Cal.App.4th 385, 388.) In
testing the sufficiency of a cause of action, a court accepts "[a]s true all
material facts properly pled and matters which may be judicially noticed but
disregard contentions, deductions or conclusions of fact or law. [A court also gives] the complaint a
reasonable interpretation, reading it as a whole and its parts in their
context." (290 Division (EAT), LLC v.
City & County of San Francisco (2022) 86 Cal.App.5th 439, 450 [cleaned

up]; Hacker v. Homeward Residential, Inc. (2018) 26 Cal.App.5th 270, 280
[“in considering the merits of a demurrer, however, “the facts alleged in the
pleading are deemed to be true, however improbable they may be”].)

Further, in ruling on a
demurrer, a court must “liberally construe” the allegations of the complaint
“with a view to substantial justice between the parties.” (See Code Civ. Proc., § 452.) “This rule of liberal construction means
that
the reviewing court draws inferences favorable to the plaintiff, not the
defendant.” (Perez v. Golden Empire Transit Dist. (2012) 209
Cal.App.4th 1228, 1238.)

In summary, “[d]etermining
whether the complaint is sufficient as against the demurrer on the ground that
it does not state facts sufficient to constitute a cause of action, the rule is
that if on consideration of all the facts stated it appears the plaintiff is
entitled to any relief at the hands of the court against the defendants the
complaint will be held good although the facts may not be clearly stated, or
may be intermingled with a statement of other facts irrelevant to the cause of
action shown, or although the plaintiff may demand relief to which he is not
entitled under the facts alleged.” (Gressley
v. Williams (1961) 193 Cal.App.2d 636, 639.)

A.

UNCERTAINTY

“[D]emurrers for uncertainty
are disfavored.” (Lickiss v.
Financial Industry Regulatory Authority (2012) 208 Cal.App.4th 1125,
1135.) “A demurrer for uncertainty will
be sustained only where the pleading is so bad that the responding party cannot
reasonably respond - i.e., [a party] cannot reasonably determine what
issues must be admitted or denied, or what counts or claims are directed
against [the party].” (Weil & Brown,
Cal. Practice Guide: Civil Procedure
Before Trial (The Rutter Group 2026) ¶ 7:85, p. 45, emphasis in original.)

As such, where a demurrer is
made upon the ground of uncertainty, the demurrer must distinctly specify
exactly how or why the pleading is uncertain, and where such uncertainty

appears by reference to page and line numbers.

(See *Fenton v. Groveland Comm. Services Dist.* (1982) 135

Cal.App.3d 797, 809 [“Generally, the failure to specify the uncertain aspects of a complaint will defeat a demurrer based on the grounds of uncertainty”].)

Although Defendants raise the demurrer in part on the ground of uncertainty, the memorandum contains no arguments regarding uncertainty and Defendants do not identify any portions of the Complaint are so faulty that Defendants cannot reasonably determine what issues must be admitted or denied, or what claims are directed against them.

Therefore, the Court declines to sustain Defendants’ demurrer on the grounds of uncertainty.

B.

FAILURE TO STATE A CAUSE OF ACTION

Defendants demur to all three causes of action on the grounds that they are barred by the applicable statute of limitations.

i.

First Cause

of Action – Medical Malpractice

Code of Civil Procedure section 340.5 provides that medical malpractice actions must be brought within one year after the plaintiff discovers, or through reasonable diligence should have discovered the injury. (Code Civ. Proc., § 340.5, emphasis added.)

Defendants argue that the Complaint alleges Plaintiff was on constructive notice of the alleged negligent injuries in October 2022, following placement of the permanent restorations, and had actual notice of the alleged injuries in February 2023, following an expert consultation with “bite specialists.” Here, Plaintiff alleges:

20. On September 29, 2022, Plaintiff received her permanent restorations under general anesthesia after six hours of exposure of prepared teeth while extensive aesthetic modifications were made to her permanent restorations, despite prior assurances that only minor adjustments would be needed. In total, this appointment lasted 720 minutes, and no option for secured try-in was provided, preventing Plaintiff from assessing function, speech, mobility and occlusion before permanent restorations were cemented.

21.From October 2022 to November 2024, Plaintiff experienced, among other symptoms, prolonged dental dysfunction, including persistent malocclusion, occlusal instability, mouth crowding, inability to control saliva, chronic pain, multiple sites of painful interference, severe lip trapping of the lower lip, and speech impairment.

22.Despite ongoing occlusal adjustments done through extensive drilling and replacement of multiple permanent restorations, complications remained unresolved.

23.Throughout this time, Defendant failed to provide a clear corrective diagnostic and treatment strategy to address complications in accordance with the standard of care expected of a reasonably skilled and careful prosthodontist.

24.Defendant repeatedly advised Plaintiff to “give it more time” and ultimately characterized the case as unprecedentedly difficult, failing to acknowledge the foreseeability of complications repeatedly raised by Plaintiff and her adult son throughout treatment.

25.In early 2023, Plaintiff requested Defendant to conduct a bite study to determine what went wrong with permanent restorations. Defendant declined, saying to give it more time.

26.Between February 2023 and March 2024, Plaintiff sought diagnostics and splint stabilization therapy from adjunct bite specialists. Findings confirmed malocclusion and occlusal dysfunction and instability. Stabilization therapy with monthly progress assessments by an adjunct TMJ bite specialist documented measurable improvement and a clear ability for Plaintiff to regain function and mobility, and to significantly improve pain and quality of life, through occlusal stabilization informed by objective diagnostics and imaging. The TMJ bite specialist’s stabilization therapy additionally provided a replicable strategy for permanent occlusal correction.

27.Defendant initially expressed a willingness to collaborate with the TMJ bite specialist to replicate Plaintiff’s appropriate occlusion with permanent restorations once determined to be stabilized and consistent.

28. When that time came in January 2024, however, Defendant instead continued to delay and obstruct Plaintiff's corrective care and indicated continued reliance on a corrective strategy based on occlusal adjustments that had proven ineffective since August 2022.

29. In August 2023, Defendant breached his duty to collaborate in Plaintiff's care by failing to attend, without explanation, a scheduled appointment with Plaintiff's periodontist, who is also Defendant's brother, to evaluate the source and appropriate treatment for Plaintiff's chronic pain, despite being visibly onsite at the time of the appointment. The periodontist attributed Plaintiff's chronic pain to persistent malocclusion, and in Defendant's absence, spent the majority of the appointment attempting to thin the top front teeth on the lingual side to reduce pain from heavy occlusal impact.

30. Defendant later dismissed the periodontist's assessment of the source of Plaintiff's chronic pain, reflecting his pattern of dismissing and questioning Plaintiff's adjunct treating specialists.

31. Despite being consistent with clinically objective treatment complications and meaningful and repetitive feedback from Plaintiff, Defendant routinely discredited findings from adjunct treaters, including bite specialists, endodontist, and periodontist.

32. On January 30, 2024, Plaintiff and her adult son presented Defendant with a detailed written summary of treatment complications, and requested a clear corrective strategy now that her occlusion was determined by the TMJ specialist to be stable in the splint and ready for permanent replication.

33. At this meeting, Defendant acknowledged his failure to achieve an appropriate occlusion, stating that he "failed to do it," but failed to provide meaningful assessment of Plaintiff's condition and a clear corrective strategy for resolution and recovery.

34. Despite his clear acknowledgement of treatment failure, Defendant deflected from Plaintiff's complications, categorizing her concerns as matters of subjective preference and dissatisfaction rather than objective clinical issues with true medical need.

35. After the January 30, 2024 meeting, Plaintiff sent multiple unanswered requests to schedule a follow-up appointment with Defendants, ultimately necessitating walking into the office on April 9, 2024 to schedule with the receptionist on duty at the time.

36. On April 17, 2024, Defendants informed Plaintiff, without prior notice or opportunity for negotiation, that continued care would be conditioned on her signing a "Release of All Claims" waiver, which included a nondisparagement clause prohibiting reporting to the Dental Board.

37. On May 16, 2024, Plaintiff and her adult son attended the appointment that she had previously walked into the office to schedule. Defendants' office staff attempted to prevent Plaintiff from seeing Defendant, stating that she would be required to sign the "Release of All Claims" waiver first. Plaintiff refused to sign the waiver in the absence of a clear corrective retreatment and diagnostic plan,

38. Plaintiff specifically requested a detailed corrective retreatment plan with all associated additional costs. Defendant discussed an ambiguous retreatment plan and continued to deflect focus to Plaintiff's satisfaction and preference rather than substantively addressing the ongoing occlusal dysfunction and clinical need,

39. It took nearly two months of repeated requests following this visit to receive Plaintiff's corrective retreatment plan totaling \$98,950. The plan provided lacked strategy and clarity, and all of Plaintiff's questions and requests for clarification remain unanswered to date.

40. Plaintiff did not sign the "Release of All Claims" and did not waive any legal rights.

41. On March 24, 2025, a board-certified prosthodontist conducted a comprehensive evaluation of Defendant's work and concluded that Plaintiff's occlusion fell below the applicable standard of care, providing a comprehensive corrective treatment plan estimated at \$116,082.

42. The evaluating prosthodontist stated that

Plaintiff's reported symptoms of a bite that is "too far forward and lateral to the left" with "multiple sites of interference" impairing speech and articulation were immediately obvious upon visual examination alone, and stated that Defendant's failure to acknowledge this constituted clear "gaslighting".

43. The Prosthodontist's clinical evaluation, further substantiated Plaintiff's reported symptoms and ongoing complaints and demonstrated the extent of the malocclusion and occlusal dysfunction, noting the following findings: Only two maxillary teeth (#6 and #12) in occlusion with no posterior support resulting in functional instability and inconsistency; Horizontal bite discrepancy with a 3mm posterior overhang on the left side and 0.5mm overhang on the right side; Anterior teeth arrangement leading to "anterior fencing effect" and significant lower lip trapping; Bilateral occlusal interferences.

44. The evaluating Prosthodontist assessed that an effective treatment sequence would incorporate deprogramming and two sets of temporary restorations, followed by permanent restorations only once a stable and consistent occlusion was demonstrated.

45. This confirmed that Defendants' failure to achieve occlusal stability and consistency before proceeding to permanent restorations constituted a significant deviation from the standard of care.

46. Defendants' repeated deflection, denial, and obstruction of necessary corrective measures foreseeably breached their professional duty, prolonged Plaintiff's suffering, and delayed her recovery.

47. Plaintiff has incurred more than \$175,000 in out-of-pocket expenses during the duration of Defendants' corrective treatment from July 2022 to November 2024, including \$141,000 paid to Defendants for what Plaintiff understood to be an all-inclusive treatment plan. Defendants later informed Plaintiff of additional charges for anesthesia and periodontal care.

(Complaint
¶¶ 20-47.)

As alleged, Plaintiff became aware

of the adverse symptoms as early as October 2022, and of the bite specialists' diagnoses as early as February 2023. Plaintiff further alleges Defendants acknowledged their own failures to Plaintiff directly in January 30, 2024. Plaintiff's suspicions that she had a claim should have been further bolstered by April 17, 2024, when Defendants informed Plaintiff that their continued care would be conditioned upon Plaintiff's release of all claims. Yet, Plaintiff did not file the instant complaint until July 8, 2025.

Plaintiff argues in opposition that her claims were tolled by virtue of Defendants' fraudulent concealment of her condition through the duration of their treatment of her, which continued through November 2024. In support, Plaintiff cites to *Sanchez v. South Hoover Hospital* (1976) 18 Cal.3d 93. There, the Supreme Court held that nondisclosure tolls the one-year limitations period applicable to medical malpractice cases. But here, Plaintiff alleges outright disclosure by Defendants on January 30, 2024 (see Complaint, ¶¶ 32-34).

Further, the *Sanchez* court found summary judgment for the defendants was proper. Noting, "when the plaintiff has notice or information of circumstances to put a reasonable person on inquiry, or has the opportunity to obtain knowledge from sources open to his investigation ... the statute commences to run[.]" the court pointed at that although the plaintiff "was dependent upon the exclusive care of defendants through March 30, 1972 [...] her reasonably founded suspicions were undeniably aroused during this period, both by her own recognition of her symptoms and by external corroboration" and thus "it is arguable that plaintiff was on notice of defendants' negligence prior to March 30." (*Sanchez v. South Hoover Hospital* (1976) 18 Cal.3d 93, 101-102.) *Sanchez* merely pointed out that certainly by the time her treatment ended, the statute had begun to run. *Sanchez* does not stand for the proposition that in no circumstances does the statute begin to run until treatment stops.

Plaintiff alleges notice of the facts underlying the potential claim as early as October 2022, when she began experiencing adverse symptoms. Further, Plaintiff sought second opinions as early as February 2023, which demonstrates actual notice, despite Defendants' alleged fraudulent concealment of her

condition. Even further, as alleged, Plaintiff had definitive actual notice of her claims by January 30, 2024, when Defendants flat out admitted the treatment failure to Plaintiff. Yet Plaintiff did not file the instant Complaint until July 8, 2025.

Therefore, the Court determines that Plaintiff's first cause of action is time-barred.

ii.

Second

Cause of Action – Negligent Infliction of Emotional Distress

"[W]hen a plaintiff asserts a claim against a health care provider on a legal theory other than professional negligence, courts must determine whether the claim is nonetheless based on the health care provider's professional negligence, which would require application of MICRA." (Larson v. UHS of Rancho Springs, Inc. (2014) 230 Cal.App.4th 336, 347.) "To make that determination, courts must examine not only the legal theory alleged, but also the nature of the health care provider's alleged conduct and the legislative history of the MICRA provision at issue." (Ibid.)

"We have repeatedly recognized that the negligent causing of emotional distress is not an independent tort, but the tort of negligence." (Burgess v. Superior Court (1992) 2 Cal.4th 1064, 1072.) "The traditional elements of duty, breach of duty, causation, and damages apply." (Eriksson v. Nunnink (2015) 233 Cal.App.4th 708, 729.)

Thus, Plaintiff's second cause of action is either not a valid cause of action in its own right and/or subject to the 1-year statute of limitations in Code Civ. Proc., § 340.5 for actions alleging professional negligence by a medical provider.

As such, the Court sustains Defendants' demurrer to Plaintiff's second cause of action.

iii.

Third

Cause of Action – Breach of Fiduciary Duty

To prevail on a claim of breach of fiduciary duty, Plaintiff must prove (1) the existence of a fiduciary duty; (2) breach of the fiduciary duty; and (3) damage proximately caused by the breach. (Jameson v. Desta (2013) 215 Cal.App.4th 1144, 1164.)

"A physician has a fiduciary duty to disclose all information material to the patient's decision when soliciting a patient's consent to a medical procedure." (Ibid.) This includes, all "personal interests unrelated to the patient's health, whether research or economic, that may affect the physician's professional judgment." (Moore v. Regents of University of California (1990) 51 Cal.3d 120, 129.)

Here, Plaintiff alleges:

3. Defendants breached this duty by 1) conditioning continued care on a "Release of All Claims" waiver that would have deprived Plaintiff of her legal and patient rights and 2) creating unreasonable and harmful delays, such as the months-long delay in scheduling a follow-up appointment after the January 30, 2024 meeting, and complicating Plaintiff's ability to receive necessary corrective treatment, despite knowing the detrimental impact to her health and daily life and 3) failing to provide a clear and objective corrective plan to appropriately address known and repetitive complications and 4) dismissing Plaintiff's repeated and specific reports of occlusal dysfunction as unsubstantiated, despite later confirmation from prosthodontic evaluation and 5) proceeding with treatment not in Plaintiff's best interest, such as continuing occlusal adjustments through drilling despite their repeated ineffectiveness and the profound emotional distress they visibly caused Plaintiff, even after her adult son's private appeal to minimize her suffering immediately upon complications with her first set of temporary restorations and 6) failing to incorporate occlusal effective stabilization methods and repeatedly reassuring Plaintiff that this would not hinder expected treatment outcome of a functional and stable occlusion.

4. By disregarding and invalidating Plaintiff's reported symptoms, Defendants not only obstructed necessary corrective care, but also undermined her ability to advocate for her own health. This conduct constituted a breach of trust and a misuse of professional authority that foreseeably prolonged Plaintiff's suffering and compounded the harm she endured.

(Complaint
at p. 14.)

With regard to “condition[ing]
continued care on a ‘Release of All Claims’ waiver,” Plaintiff alleges that by
that point, Defendants had disclosed their treatment failures to her
three months prior. Further, Plaintiff
alleges that she did not actually sign the waiver.

With respect to the other alleged
breaches, Plaintiff does not establish a fiduciary duty not to delay treatment,
provide a clear corrective plan, dismiss Plaintiff’s concerns, proceed with a
treatment that is not in Plaintiff’s best interest, or to incorporate occlusal
effective stabilization methods. Rather,
those breaches would all constitute medical negligence, not a breach of any
fiduciary duty.

Therefore, the Court sustains
Defendants’ demurrer to the third cause of action.

2. LEAVE TO AMEND

A plaintiff has the burden of showing in what manner the
complaint could be amended and how the amendment would change the legal effect
of the complaint, i.e., state a cause of action. (See *The Inland Oversight
Committee v. City of San Bernardino* (2018) 27 Cal.App.5th 771, 779; *PGA
West Residential Assn., Inc. v. Hulven Int’l, Inc.* (2017) 14 Cal.App.5th
156, 189.) A plaintiff must not only state the legal basis for the amendment,
but also the factual allegations sufficient to state a cause of action or claim.
(See *PGA West Residential Assn., Inc. v. Hulven Int’l, Inc.*, *supra*,
14 Cal.App.5th at p. 189.) Moreover, a plaintiff does not meet his or her
burden by merely stating in the opposition to a demurrer or motion to strike
that “if the Court finds the operative complaint deficient, plaintiff
respectfully requests leave to amend.” (See *Major Clients Agency v Diemer*
(1998) 67 Cal.App.4th 1116, 1133; *Graham v. Bank of America* (2014) 226
Cal.App.4th 594, 618 [asserting an abstract right to amend does not satisfy the
burden].)

Here, Plaintiff

has failed to meet this burden as Plaintiff does not address whether leave should be granted if the demurrer is sustained.

CONCLUSION AND ORDER

For the reasons stated, the Court sustains Defendants' demurrer to the Complaint in its entirety without leave to amend.

Further, the Court will enter the proposed Order lodged on October 23, 2025 in conformity with the Court's ruling, and orders Defendants to lodge a proposed Judgment in conformity with the Order on or before August 12, 2026.

Defendants shall provide notice of the Court's ruling and file the notice with a proof of service forthwith.

DATED: July 29, 2026 _____/s/_____

Michael
E. Whitaker

Judge
of the Superior Court